

A male sling is a soft strip of mesh placed under the urethra (the tube urine flows through). It treats urine leakage, most often the leakage with coughing, lifting or moving that can follow prostate surgery. Unlike an artificial sphincter, there is nothing to operate.

About This Procedure

Leaking urine when you cough, lift or move is called **stress incontinence**. A sling treats it by placing a mesh strip under the urethra and lifting it back toward its normal position.

The sling **supports** the urethra rather than squeezing it shut. Once you heal, you **urinate normally**. There is no pump or button.

Benefits and Risks in Numbers

about
7 in 10

men are dry or much better 4 to 5 years later

1–2 in 10

have short-term trouble urinating, which usually settles within a week

about
1 in 10

need another operation later

Numbers are averages from published studies. Your own risk may differ; ask your team.

What to Know Before You Decide

- A sling works best for **mild to moderate** leakage in men whose own sphincter still works. Your surgeon checks this first.
- For heavier leakage, or after **pelvic radiation**, an **artificial urinary sphincter** usually works better.
- A sling may reduce leakage rather than stop it. If it is not enough, a sphincter can still be placed later.

Learn the Terms

Stress incontinence

Leaking urine when you cough, lift or move.

Urethra

The tube that carries urine out of the body.

Sling

A soft strip of mesh placed under the urethra to lift and support it.

Sphincter

The muscle ring that holds urine in.

Repositioning check

A look inside with a small camera to see if lifting the urethra is likely to help.

Urinary retention

Trouble emptying the bladder. After a sling it is usually short-term.

Artificial sphincter (AUS)

A different device, with a pump, usually used for heavier leakage.

Will it hurt? You will be asleep or numb from the waist down, so you feel nothing during surgery. Afterward, expect soreness and bruising in the groin and behind the scrotum. Pain usually settles within about 4 weeks. Many people go home the same day.

First days

Soreness and bruising. Ice and pain medicine. You may need a catheter briefly.

First weeks

No heavy lifting, straining, bike riding or sex until your team says.

Weeks to months

Control often keeps improving. Use pads until you know.

Follow-up visit

Your team checks how well it is working.

How to Get Ready

- Follow your surgery instructions about eating, drinking and which medicines to stop, including blood thinners.
- A **urine test** checks for infection, which is treated first. A **camera check** shows whether a sling is likely to help.
- **Stop smoking.** Keep diabetes under good control. The sling is a mesh implant, so preventing infection matters.

Tell your team ahead of time if you:

- Have had **pelvic radiation**
- Have had earlier leakage surgery, or a narrowing of the urethra
- Have any infection, anywhere

What Happens During Surgery

- 1 You get general or spinal anesthesia and antibiotics.
- 2 Through small cuts in the groin and behind the scrotum, the surgeon passes the sling under the urethra and lifts it into place.
- 3 The cuts are closed. You may have a catheter for a short time.

Call the office that did your surgery if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Redness, swelling or fluid leaking from a cut
- Groin or genital pain that is getting worse, not better
- A weak stream or trouble emptying your bladder

Go to an emergency department if:

- You cannot urinate for several hours and your bladder feels full or painful
- You cannot reach the office and are worried

Three Things to Remember

1. A sling lifts and supports the urethra. There is nothing to operate.
2. It works best for mild to moderate leakage in men who have not had radiation.
3. If a sling is not enough, an artificial sphincter can still be placed later.